

LOW MOOD, STRESS OR ANXIETY

Give urgent attention to the patient with suicidal thoughts or behaviour ↪ 83.

Assess the patient with low mood, stress or anxiety. If patient known with depression, rather give routine depression care → 144.

Assess	Note
Anxiety	- If excessive worry causes impaired function/distress for at least 6 months with ≥ 3 of: muscle tension, restlessness, irritability, difficulty sleeping, poor concentration, tiredness: generalised anxiety disorder likely ↪ 144. - If anxiety is induced by a particular situation/object (phobia) or is repeated sudden fear with physical symptoms and no obvious cause (panic), discuss/refer. - If anxiety > 1 month linked to a bad experience, with ≥ 3 of: 1) nightmares or flashbacks 2) avoids situations/people 3) constantly on guard, or easily startled 4) feels numb or detached from others/ surroundings, post-traumatic stress disorder (PTSD) likely, discuss/refer.
Depression	If not already done: in the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either → 143.
Alcohol/drug use	In the past year, has patient: 1) drunk ≥ 4 drinks ¹ /session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↪ 142.
Abuse	If patient is being abused ↪ 88.
Stressors	- Help identify psychosocial stressors. Ask about family or relationship problems, infertility, financial difficulty, bereavement, chronic ill-health. If sexual problems ↪ 58. - If patient has a life-limiting illness, also consider giving palliative care ↪ 170. - If older person: ask about loneliness and if available, refer to nearest social club for older people in the area.
Women's health	- If recent delivery: give postnatal care ↪ 164 and if available, refer to mother's support group. - If woman > 40 years, ask about menopausal symptoms: hot flushes, night sweats, vaginal dryness, mood changes, difficulty sleeping and sexual problems ↪ 169.
Medication	Review medication: prednisone, efavirenz, metoclopramide, theophylline and contraceptives can cause mood changes. Discuss with doctor. Consider alternative contraceptive ↪ 154.

Advise the patient with low mood, stress or anxiety

Health for All

↪ 104

- Encourage patient to question negative thinking and be realistic if s/he often predicts the worst, generalises, exaggerates problem, inappropriately takes the blame, takes things personally.
- Help the patient to choose strategies to get help and cope:

Get enough sleep

If patient has difficulty sleeping, give advice ↪ 87.



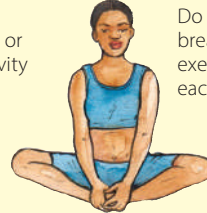
Encourage patient to take time to relax:



Spend time with supportive friends or family.



Find a creative or fun activity to do.



Do a relaxing breathing exercise each day.



Get active

Regular exercise might help.



Access support

Link patient with helpline or support group ↪ 178.

Limit alcohol and avoid drugs

- Limit alcohol to ≤ 2 drinks/day and avoid alcohol on at least 2 days/week.
- Avoid drugs.



- If stressors identified, discuss possible solutions. If needed, refer to available counsellor, psychiatric nurse/psychologist or social worker.
- Deal with bereavement issues if patient or family member has a life-limiting illness or if patient is recently bereaved:
 - Acknowledge grief reactions: denial, disbelief, confusion, shock, sadness, bargaining, yearning, anger, humiliation, despair, guilt and acceptance.
 - Allow patient/family to share sorrow and talk of memories, the meaning of the patient's life or religious beliefs. Suggest connecting with a spiritual counsellor as appropriate.
 - Identify worrying issues (e.g. child care, will and funeral arrangements) and who can give practical support with these before and after the patient dies.
- For tips on how to communicate effectively ↪ 176.

Offer to review the patient in 1 month.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer.